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Medical

ALCOHOL AND DRUG ABUSE PREVENTION AND TREATMENT (ADAPT) PROGRAM

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This publication implements Air Force Policy Directive 44-1, *Medical Operations*. It establishes guidance for the Department of the Air Force (DAF) Alcohol and Drug Abuse Prevention and Treatment (ADAPT) Program. This instruction provides guidance for the identification, treatment and management of personnel with substance use disorders (SUDs) and describes DAF policy regarding alcohol, illicit drug use, and prescription drug misuse, as well as other addictive behaviors. This instruction applies to all active-duty United States Space Force, Regular Air Force, Air Force Reserve, and Air National Guard personnel whenever eligible for Department of Defense (DoD) medical services. The Air Reserve Component (ARC) does not maintain separate systems to provide mental health treatment, including SUD treatment. Clarification about ARC-specific policies, processes, and/or procedures should be directed to Headquarters Air Force Reserve Command (AFRC)/SG's Mental Health Consultant at Robins AFB, GA for reserve-related inquiries or the Air National Guard (ANG)/SG's Medical Service Office at Joint Base Andrews, Maryland. When not eligible for DoD medical services, ARC members will obtain, when needed, non-military SUD services at their own expense.

This Instruction requires the collection and or maintenance of information protected by the Privacy Act of 1974 authorized by Title 5, United States Code (U.S.C.) Section 552a.Title 10, U.S.C. 9013., 42 U.S.C.290dd-2, et seq., and Executive Order 9397, Numbering System for Federal Accounts Relating to Individual Persons, November 22, 1943 as amended by Executive Order 13478, Amendments to Executive Order 9397 Relating to Federal Agency Use of Social Security Numbers, November 18, 2008. The applicable System of Records Notice (SORN) F044 SG S,

Alcohol and Drug Abuse Prevention and Treatment Program, applies and is available at: **http://dpclo.defense.gov/Privacy/SORNs.aspx**. Refer recommended changes and questions about this publication to the Office of Primary Responsibility (OPR) listed above using DAF Form 847, *Recommendation for Change of Publication*; route DAF Form 847 from the field through the appropriate chain of command. This publication may be supplemented at any level, but all supplements must be routed to the OPR listed above for coordination prior to certification and approval. The authorities to waive wing, unit and delta level requirements in this publication are identified with a Tier ("T-0, T-1, T-2, T-3") number following the compliance statement. See DAF Manual (DAFMAN) 90-161, *Publishing Processes and Procedures*, for a description of the authorities associated with the tier numbers. Submit requests for waivers through the chain of command to the appropriate tier waiver approval authority. For non-tiered compliance items in this publication, submit requests to the OPR of this publication. Ensure all records generated as a result of processes prescribed in this publication adhere to Air Force Instruction 33-322, *Records Management and Information Governance Program*, and are disposed in accordance with the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System.

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Chapter 1

ROLES AND RESPONSIBILITIES

1.1. Air Force Surgeon General (AF/SG). AF/SG oversees policy and implementation of the Department of the Air Force (DAF) Alcohol and Drug Abuse Prevention and Treatment (ADAPT) Program.

1.2. Air Force Medical Command, Mental Health Branch (AFMEDCOM/A3HM).

1.2.1. Develops, implements, and manages ADAPT operations to support established readiness policies.

1.2.2. Manages programming and execution of the DAF ADAPT budget.

1.2.2.1. Will ensure ADAPT receives adequate funding to support education, prevention, and outreach efforts. **(T-1)**

1.2.2.2. Will allocate funding available for continuing education and training for Certified Alcohol and Drug Counselors (CADCs) to meet International Certification and Reciprocity Consortium (IC&RC) requirements. **(T-0)**

1.2.3. Advises AF/SG on development of detailed guidance to ensure standardization and compliance with DAF, Defense Health Agency (DHA), and Department of Defense (DoD) management of substance use requirements.

1.2.4. Develops and prioritizes inspection requirements for this instruction IAW DAFI 90-302, *The Inspection System of the Department of the Air Force.*

1.2.4.1. Coordinates with Headquarters Air Force and Field Operating Agencies involved in ADAPT.

1.2.5. Communicates with federal and civilian agencies that have shared or overlapping equity in ADAPT responsibilities and interests.

1.2.6. Serves as DAF representative on the Substance Misuse & Addictive Behaviors Working Group and the Addictive Substance Misuse Advisory Council on DAF-related substance misuse trends and policy requirements.

1.2.7. Convenes and attends conferences and other professional forums that address ADAPT-related topics or emerging issues and determines appropriate DAF representation at these events.

1.2.8. Advises on ADAPT manpower and personnel issues.

1.2.9. Responds to ADAPT-related complaints and suggestions, Congressional and interagency inquiries, and Freedom of Information Act requests.

1.2.10. Serves as Office of Collateral Responsibility on drug deterrence.

1.2.11. Monitors the training requirements for CADCs and CADC candidates.

1.2.12. Develops procedures for managing and documenting ADAPT activities related to readiness oversight and support.

1.2.13. Develops and prepares ADAPT statistical data and reports for program management and policy development.

1.2.14. Collects data for ADAPT and prepares reports as required by DoD, DHA, and DAF higher headquarters.

1.2.15. Reviews inspection reports and other assessments. Reports trends and recommends process improvements to the field.

1.2.16. Disseminates DoD, DHA, DAF, and career field policies and ADAPT requirements to Major Command (MAJCOM) and Medical Readiness Command (MRC) Mental Health Leaders.

1.3. The Judge Advocate General of the Air Force (AF/JA). AF/JA provides legal opinions, instructions, guidance, and assistance regarding ADAPT program and policies.

1.4. Headquarters Air Force Director of Security Forces. Ensures that Security Forces provides Law Enforcement sensitive data to Installation ADAPT Program Managers (PM) and/or MDG-appointed designees on alcohol or other drug related misconduct incidents.

1.5. MAJCOM Surgeon Generals, Direct Reporting Units (DRU), and MRC.

1.5.1. Ensures that each installation in the command establishes and maintains ADAPT capabilities IAW DoD, DHA, and DAF policies.

1.5.2. MAJCOM SG and DRUs will appoint a Mental Health Leader and a 4C0X1, Mental Health Service, MAJCOM Functional Manager (MFM).

1.6. Mental Health Leaders, MRC and MAJCOM. Commander designated mental health subject matter experts, traditionally a licensed mental health provider and a 4C0X1 senior enlisted member, assigned at the MAJCOM SG or MRC headquarters.

1.6.1. Identifies and corrects ADAPT service delivery issues that cannot be resolved at the installation level.

1.6.2. Refers ADAPT-related issues that cannot be resolved at the MTF or Medical Wing level to the DHA Substance Use Disorder Clinical Care (SUDCC) Program and the A3HM ADAPT Chief and Manager.

1.6.3. Ensures installation mental health providers and technicians working in ADAPT receive formal continuous education, training, guidance, and that certification processes are performed IAW DAF policy.

1.6.4. Responds to applicable substance misuse related complaints, suggestions, and congressional and/or higher-level inquiries.

1.6.5. MAJCOM Mental Health Leaders will conduct periodic ADAPT Staff Assistance Visits at the request of the Medical Commander to ensure program fidelity and appropriate enlisted training and utilization.

1.6.6. Advises Mental Health Flight leaders on effective training programs, technician utilization, and budget execution related to the ADAPT program.

1.6.7. Disseminates DoD, DHA, DAF, and career field policies and ADAPT requirements to respective flight leaders.

1.6.8. In addition to the above duties, the 4C0X1 MFM will:

1.6.8.1. Monitor the enlisted health and manning of their respective ADAPT programs and elevate concerns to Air Force Personnel Center and the 4C0X1 Career Field Manager. **(T-1)**

1.6.8.2. Coordinate and manage command training and enlisted personnel issues across their respective ADAPT programs. **(T-1)**

1.6.8.3. Liaise with installation ADAPT PM, unit commanders, and the Air Force Substance Abuse Counselor Certification Board (AFSACCB) regarding any ethical violations or concerns. **(T-1)**

1.6.8.4. Serve as a voting member to the AFSACCB. **(T-1)**

1.7. Installation Commander.

1.7.1. Responsible for the installation ADAPT program.

1.7.2. Administers and monitors the installation ADAPT program.

1.7.3. Will ensure allocation of adequate space for provision of substance misuse education and treatment services. **(T-3)**

1.7.4. Will authorize Installation ADAPT PM to receive Law Enforcement Sensitive information identifying all alcohol and drug-related misconduct. **(T-1)**

1.7.5. Ensures naloxone reversal availability on the installation IAW Public Law and DODI 1010.04, *Problematic Substance Use and Gambling Disorder.*

1.8. Installation Defense Force Commander. Ensures the ADAPT PM has daily access to Law Enforcement Sensitive investigative or Privacy Act information to identify Alcohol-Related Misconduct.

1.9. Geographically Separated Unit (GSU) Commanders. GSU Commanders will refer Service members to the nearest Military Medical Treatment Facility (MTF) with a substance use program for assessment when substance misuse is suspected to be a contributing factor in misconduct or when an individual is suspected of having a problem with alcohol or other substances. **(T-1)** Treatment of SUDs for patients assigned to GSUs may include services through civilian resources with on-going case-management provided through the ADAPT PM at the nearest MTF unless services are available via tele-health or another more practical means provided by an MTF.

1.10. Air Reserve Component (ARC) Commanders.

1.10.1. Unit Commanders are encouraged to place members suspected of having a SUD on duty status to receive the initial assessment and recommendations for treatment from the ADAPT program IAW DAFMAN 48-123, *Medical Examinations and Standards*, and DAFMAN 36-2136, *Reserve Personnel Participation.*

1.10.2. For members not placed on duty status, the Commander will refer members suspected of having a SUD to a non-MTF for evaluation and treatment of a SUD. The non-military provider must be a licensed mental health provider or licensed and/or certified substance abuse counselor. ARC members who have not been entered into the ADAPT program will ensure the civilian SUD records are available for review by the Reserve Medical Unit or the Guard Medical Unit. ARC members who have not been entered into formal treatment may receive support services and enhanced coordination from the local or regional Director of Psychological Health (DPH) aligned with the ARC unit. ADAPT staff will coordinate warm hand-off with ARC DPH for additional care coordination or support. **(T-0)**

1.11. MDG Commander.

1.11.1. Serves as the Office of Primary Responsibility for SUD issues.

1.11.2. Will appoint a privileged mental health provider as the ADAPT PM by assigning the duty title consistent with the Air Force Medical Service flight path. When the Mental Health Provider is not licensed, a licensed privileged clinical supervisor will be required to co-sign those functions designated IAW Department of Defense Instruction (DoDI) 1010.04. **(T-0)**

1.11.3. Ensures substance use treatment is conducted IAW DHA and DAF guidance.

1.11.4. Will ensure all medical personnel, working in direct patient care, or clinical supervisory roles receive annual training in SUDs and chemical dependency IAW DoDI 1010.04 and Defense Health Agency Administrative Instruction (DHA-AI) 6025.26, *Management of Substance Use Disorder*. (see **Table 2.1**). **(T-1)**

1.11.5. Ensures timely aeromedical evacuation of eligible beneficiaries on inpatient status for treatment of SUDs.

1.11.6. Appoints a MTF Addiction Champion to oversee substance use medical support activities IAW DHA-AI 6025.26. **(T-0)**

1.12. ADAPT Program Manager (PM).

1.12.1. Manages the installation ADAPT program IAW current DoD, DHA, and DAF policies, and provides clinical supervision to the ADAPT staff.

1.12.2. Must be a privileged mental health provider. **(T-0)**

1.12.3. Attends ADAPT PM training prior to, or within six (6) months of, assuming ADAPT PM duties. **(T-2)**

1.12.4. Will coordinate clinic resources to provide effective substance use-related prevention, education, identification, assessment and treatment. **(T-0)**

1.12.5. Ensures ADAPT budget resources are properly used for CADC continuing education, as well as community prevention and outreach. Requests Defense Health Program (DHP) funds from the local MTF resource management office for expenses related to patient treatment and education, clinical supplies, and staff development.

1.12.6. Assists commanders, First Sergeants and supervisors with identifying and referring individuals needing substance use education and/or treatment services.

1.12.7. Chairs Treatment Team Meetings.

1.12.8. Ensures non-privileged ADAPT staff receive continuous SUD training and are appropriately maintaining certification requirements or are actively participating in the certification process.

1.12.9. Manages clinical practice of CADCs. To ensure ongoing training and competency assessment for CADCs, the ADAPT PM, or designee, must perform direct observation of the CADC while performing individual or group treatment at least two times per month for a total of at least two hours monthly. Direct observation involves the monitoring of a counselor's clinical skills by a supervisor to allow for immediate feedback and guidance to ensure competency, adherence to ethical guidelines, and the delivery of effective, safe patient care. Competency assessment will focus on standards defined within the Four Performance Domains of the Substance Abuse Counselor and should consider the full spectrum of competency within these standards. All supervision activities will be documented in the CADC's official Air Force training record. **(T-1)**

1.12.10. Ensures development and implementation of ADAPT education (**Table 2.1**) and prevention programs.

1.12.11. Conducts required reviews of the patient's medical records, and all documentation provided by the ADAPT staff.

1.12.12. Ensures appropriate referrals are ordered for medical, psychiatric, or laboratory examinations as needed based on diagnostic assessment.

1.12.13. Assists all Service members across ARC-led installations and GSUs with ADAPT-related issues as outlined in the local installation host-tenant support agreement or memorandum of understanding.

1.12.14. Coordinates with Active Component and ARC DPH to educate installation leaders, Community Action Team (CAT) members, and base population on ADAPT capabilities.

1.12.15. Ensures ARC members who have been evaluated by ADAPT staff but are not eligible for DoD medical services are referred for necessary education or supported in securing treatment through a non-military provider with warm hand-off completed with ARC DPH.

1.12.16. Collaborates and communicates with ARC personnel, to include commanders, Reserve Medical Unit/Guard Medical Unit military providers and ARC DPH, for each ARC member who presents to ADAPT for evaluation and services. ADAPT staff will:

1.12.16.1. Provide all ARC members assessment, psychoeducation, and non-medical services. **(T-0)**

1.12.16.2. Provide medical treatment for ARC members eligible for benefits and within commuting distance to an MTF IAW TRICARE guidelines for access. **(T-0)**

1.12.16.3. Provide virtual monitoring of treatment for members being seen by a non-DoD provider. **(T-0)**

1.12.17. Will provide fitness for duty and mobility status recommendations to ARC Commanders for ARC members who have been referred by the ADAPT PM and seen by non-military providers for SUD evaluation and/or treatment. **(T-1)**

1.12.18. Coordinates with non-military providers for ARC members who have been referred by ADAPT staff to non-military providers for treatment to ensure treatment records are available to the ADAPT staff. The member will ensure the records are available for review by the ADAPT provider for status recommendations as determined by the Commander, in consultation with an ADAPT provider. The member must be on duty status to be seen in ADAPT for any appointments associated with status recommendations. **(T-1)**

1.12.19. Will provide Personnel Reliability Assurance Program (PRAP), and Flying status recommendations IAW DAFMAN 13-501, *Nuclear Weapons Personnel Reliability Program (PRP)*, DAFI 31-117, *Arming and Use Of Force*, AFMAN 48-149, *Flight And Operational Medicine Program (FOMP),* and other applicable regulations. **(T-1)**

1.12.20. Will collaborate with the MTF Addiction Champion to ensure medical oversight of ADAPT program patients and SUD care coordination IAW clinically indicated interventions and American Society of Addiction Medicine-Patient Placement Criteria (ASAM-PPC). **(T-1)**

1.13. Certified Alcohol and Drug Counselor (CADC). DAF personnel who possess a formal certification in addiction counseling may serve in clinical roles as CADCs within the ADAPT program. Government Service and contract personnel hired to perform addiction counseling duties are subject to the certification maintenance requirements of their certifying authority.

1.13.1. Under the authority of the IC&RC, the AFSACCB oversees initial certification and certification maintenance requirements for all Active Duty CADCs within the DAF. The AFSACCB retains the authority to revoke or suspend an Active Duty CADC's certification for just cause IAW the *Air Force Substance Abuse Counselor Certification Board Governance Policy Manual.*

1.13.1.1. Education and Certification/Recertification Requirements:

1.13.1.1.1. Eligibility for certification. Air Force personnel who are currently performing duties related to ADAPT and meet minimum certification requirements as defined by the AFSACCB are eligible to apply for certification. Certification requirements are listed in the *Air Force Substance Abuse Counselor Certification Handbook* located on the ADAPT Knowledge Exchange site.

1.13.1.1.2. Sign the USAF CADC Code of Ethics affirming allegiance to a system of principles relating to their moral duty and obligations.

1.13.1.1.3. Pass a recognized written examination administered by the Air Force or another IC&RC sanctioned board.

1.13.1.1.4. Verify that CADC credential information is entered into the Centralized Credentials Quality Assurance System or alternate system approved by the local credentialing office.

1.13.1.2. Maintaining Certification. CADCs certified under the AFSACCB must recertify every three (3) years IAW the Air Force Substance Abuse Counselor Certification Handbook. Enlisted Airmen certified through the AFSACCB will maintain their certification while holding the 4C0X1 Air Force Specialty Code (AFSC). **(T-1)** CADCs are responsible for ensuring they maintain their competencies and continuing education requirements.

1.13.1.2.1. Individuals initially certified by the AFSACCB who are in a special duty, cross-flowed or cross-trained into another career field, can be recertified if all requirements are met. Members in this status are responsible for all financial responsibilities related to certification maintenance and will not be funded by DAF ADAPT. **(T-1)**

1.13.1.2.2. Expired Certification. CADCs that do not recertify before their credential expires will be placed in an "Inactive" status. Upon initiation of the "Inactive" status, the CADC will function as a non-CADC mental health technician until certification is reinstated. Certifications that have been expired for more than sixty (60) days will be revoked and are subject to AFSC disqualification criteria IAW the 4C0X1 Air Force Enlisted Classification Directory. **(T-1)**

1.13.2. Scope of Practice/Supervision. CADCs may perform functions as outlined in the Department of the Air Force Enlisted Classification Directory (DAFECD) and may perform these functions with indirect supervision, as authorized by the ADAPT PM. For initial assessment, treatment plan development or modification, and crisis intervention activities, a licensed privileged mental health provider must perform "eyes on" supervision of CADCs. "Eyes-on" supervision consists of the mental health provider meeting with the patient for a sufficient duration and assessment to determine the diagnosis, the appropriateness of the treatment plan and to address crises to ensure safety. **(T-0)**

1.13.2.1. Responsibilities include oversight and supervision of non-CADC mental health technicians to develop the skills and competencies required for certification.

1.13.2.2. Removal of CADC from duties due to adverse actions and reporting details regarding the adverse actions to the AFSACCB for a formal review of the member's certification eligibility.

1.13.3. Non-certified 3-level mental health technicians require skills development and educational oversight while performing in an ADAPT setting as development of CADC competencies is fundamental for all mental health technicians.

1.13.3.1. Non-CADC 3-level Mental Health Technicians may work within the 4C0X1 DAFECD scope of practice when supervised by a CADC or licensed privileged mental health provider and must have direct supervision during the entire ADAPT patient contact.

1.13.3.2. A licensed privileged mental health provider is responsible for "eyes-on" evaluation of the patient during the patient visit. The privileged mental health provider who performed this supervision must co-sign the note in the patient record. **(T-0)**

1.13.3.3. All non-CADC technicians and their supervisors must have a written plan focused on developing Substance Abuse Counselor competencies based on current skill level requirements. **(T-1)** Topics could include but are not limited to: American Society of Addiction Medicine, Four Domains/12 Core Functions, Motivational Interviewing, and Ethics.

Chapter 2

THE ADAPT PROGRAM

2.1. Alcohol Misuse. Alcohol misuse negatively affects individual behavior, duty performance, and/or physical and mental health. The DAF provides comprehensive clinical assistance to active-duty Service members and will support referral coordination for other eligible beneficiaries seeking help for an alcohol problem. Commanders should encourage members to seek help for problems with alcohol without fear of negative consequences.

2.2. Illicit Drug Use. The DAF does not tolerate the illegal use of illicit substances, including the wrongful use of over the counter or prescription medications by Airmen or Guardians. Such use is a serious breach of discipline; is incompatible with military service; automatically places the member's continued service in jeopardy; and can lead to criminal prosecution resulting in a punitive discharge or administrative actions, including separation or discharge under other than honorable conditions.

2.3. Addictive Behaviors. The DAF recognizes that addiction diagnoses such as gambling disorder and other non-substance addictive behaviors are problematic, may impact readiness, and should be treated as a mental health concern. Non-substance related addictive behaviors will be treated in the Mental Health Clinic. Command notifications for non-substance related addictive behaviors will be performed IAW DoDI 1010.04 and AFI 44-172 requirements. **(T-0)** Members being treated solely for an addictive behavior concern or disorder are not required to enroll into the ADAPT program.

2.4. ADAPT Overview.

2.4.1. The primary objectives of the ADAPT program are to:

2.4.1.1. Promote readiness, health, and wellness through the prevention and treatment of substance misuse.

2.4.1.2. Minimize the negative consequences of substance misuse, to the individual, family, and organization.

2.4.1.3. Provide comprehensive education and treatment to individuals who experience problems attributed to substance misuse.

2.4.1.4. Restore function and return members to unrestricted duty status, or to assist them in their transition to civilian life, as appropriate.

2.4.2. These objectives are met through three (3) levels of activities:

2.4.2.1. Primary Prevention and Education: This includes population-based outreach, education, prevention programs, screening, and consultation. Community-based prevention and education efforts will be delivered by ADAPT staff through coordinated efforts with other community agencies. Clinical services, screening and consultation will be delivered through the ADAPT program. **(T-3)**

2.4.2.2. Secondary Prevention: This is indicated for members engaging in problematic alcohol use but have not yet developed problems associated with their drinking. Individuals in this group can be identified through screening in primary care or other appropriate settings. This involves global screenings for alcohol misuse, as well as initiatives to prevent future drug or alcohol misuse, prescription drug misuse, or drug use with individuals who are identified as high risk or are suspected of substance misuse. Selective prevention includes screening, assessment, education, brief preventive counseling, and tailored feedback in specific individuals or groups identified as moderate to high risk. Individuals or groups identified as moderate to high risk for problematic alcohol use based on installation trends and/or adverse health risk will be provided Alcohol Brief Counseling (ABC) IAW the ABC Manual. **(T-1)**

2.4.2.3. Treatment: IAW Clinical Practice Guidelines (CPGs), this involves providing evidence-based SUD treatment for individuals who are misusing or are dependent on alcohol or drugs. Installations will provide substance use care IAW DHA-AI 6025.26, *Management of Substance Use Disorders*. **(T-0)** Other higher-level care requirements can be provided by military or civilian facilities.

2.5. Information Disclosures.

2.5.1. ADAPT treatment information is collected and maintained as a part of active treatment or aftercare services IAW 42 U.S.C. 290dd-2 and AFI 33-332, *Air Force Privacy and Civil Liberties Program*. These records are protected from public disclosure and are released only under circumstances listed in 42 U.S.C. 290dd-2(b) and (c). **(T-0)**

2.5.2. ADAPT treatment information may be disclosed or released to other offices or agencies within the Armed Forces or disclosed to components of the Department of Veteran's Affairs furnishing healthcare to veterans.

2.5.3. Disclosures related to child abuse or neglect will be reported to appropriate civilian or military authorities to the extent necessary to comply with military, state, or local reporting requirements. **(T-0)**

2.6. Eligibility.

2.6.1. All beneficiaries are eligible for treatment, following TRICARE guidelines for access. Eligible beneficiaries will receive SUDCC services as offered through their selected health care option. **(T-0)**

2.6.2. Members pending administrative separation following ADAPT failure will maintain SUD service eligibility within the ADAPT program. **(T-0)**

2.6.3. Civilian employees requiring assessment and/or treatment of a SUD who do not otherwise meet eligibility criteria for services within the MTF will be referred to the Employee Assistance Program IAW DHA-AI 6025.26. **(T-1)**

2.7. Substance Misuse Prevention Strategies.

2.7.1. In collaboration with the CAT and Installation DPH, ADAPT develops and implements prevention programs geared toward increasing organizational and individual awareness of substance-related issues IAW DAFI 90-5001, *Integrated Resilience*. Substance misuse prevention efforts enhance unit and individual resilience, both of which can be compromised by hazardous alcohol use and SUDs. Prevention strategies must be comprehensively structured to educate and inform the overall population as well as specifically target higher risk populations. **(T-0)** Substance misuse prevention and education programs may be tailored to meet the specific needs of the organization.

2.7.2. The ADAPT PM will coordinate with the installation DPH to ensure all substance use-related prevention activities are conducted IAW DoDI 1010.04 requirements. All trainings detailed in **Table 2.1** are required annually for each specified population. **(T-0)**

Table 2.1. Substance Use Education.

	Target Audience	Learning Objectives
1	All DAF personnel (Active and Reserve Components)	a. Identify reasons for use of alcohol, tobacco, drugs, and gambling, and associated internal and external pressures. b. Examine the consequences of alcohol, tobacco, drug use, and gambling, including impact on personal life and force readiness. c. Identify signs, symptoms, and risk behaviors in self and others related to SUDs. d. Describe how substances are used as coping mechanisms. e. Identify and utilize alternatives to alcohol and tobacco use. f. Ability to recognize and adhere to SUD-related policies and critically examine substance misuse messaging from peers and media.
2	Squadron, Group, and Wing/Delta-Commanders (or equivalent) and First Sergeants	a. Current DAF and DoD policies and procedures related to SUD and gambling disorder. b. Early identification and management of problematic substance use within their units. c. Referral processes for personnel displaying signs or symptoms of problematic substance use. d. Prevention and treatment resources available to assigned personnel. e. Roles and responsibilities for command teams within the ADAPT setting. f. Interpretation of organizational outcome measures and prevention strategies related to SUDs.
3	Mental health providers, medical providers, substance use counselors, and nursing staff credentialed in recognizing substance use and gambling disorders	a. Identification of medical policies and processes related to problematic substance use and gambling disorder. b. Identification of signs, symptoms, and risky behaviors to self or others related to SUDs and gambling disorder. c. Identification of signs and symptoms of co-occurring disorders. d. Prevention processes for SUDs, gambling disorder, and tobacco use. e. Interpretation of healthcare outcome measures inherent to the prevention of SUD, gambling disorder, and tobacco use.

2.8. Referrals.

2.8.1. Self-Referral. Airmen and Guardians with substance use concerns are encouraged to seek assistance from their unit Commander, First Sergeant, a military medical professional, or mental health provider. Self-identification is reserved for members who are not currently under investigation or pending action based on drug- or alcohol-related misconduct.

2.8.1.1. Self-identified members who enter the ADAPT assessment process will be held to the same standards as others entering substance use education, counseling, and treatment programs. **(T-0)** Informed consent disclosure requirements will be maintained IAW section 2.11. of this instruction. For members not diagnosed with a SUD, the member's command team will not be notified. **(T-0)**

2.8.1.2. Airmen and Guardians may voluntarily disclose evidence of alcohol-related misconduct, personal drug use, or possession to the Commander, First Sergeant, ADAPT staff, or another military medical professional.

2.8.1.3. Commanders will grant limited protection for members who reveal information listed in section 2.8.1.2. with the intention of obtaining treatment for a substance use disorder or concern. **(T-0)**

2.8.1.4. Commanders will not use voluntary disclosure against a member in an action under the Uniform Code of Military Justice or when weighing characterization of service in a separation. **(T-0)**

2.8.1.5. Disclosure is not voluntary if the member has previously been:

2.8.1.5.1. Apprehended for drug involvement or a UCMJ offense involving alcohol-related misconduct.

2.8.1.5.2. Placed under investigation for illicit drug use or a UCMJ offense involving alcohol-related misconduct. The determination of "placed under investigation" status is made on the circumstances of each individual case. Consult with DHA Network legal consultant or other legal and/or investigative authorities regarding any questions.

2.8.1.5.3. Ordered to give a urine sample as part of the drug-testing program in which the results are still pending or have been returned as positive.

2.8.1.5.4. Advised of a recommendation for administrative separation for illicit drug use or for a basis involving alcohol-related misconduct.

2.8.1.6. The limited protection under this section for self-identification does not apply to:

2.8.1.6.1. The introduction of evidence for impeachment or rebuttal purposes in any proceeding in which the evidence of illicit drug use (or lack thereof) or evidence of alcohol-related misconduct (or lack thereof) has been first introduced by the member.

2.8.1.6.2. Disciplinary or other action based on independently derived evidence (other than the results of commander-directed drug testing), including evidence of continued illicit drug use or alcohol-related misconduct after the member initially entered the treatment program.

2.8.2. Commander Referral.

2.8.2.1. Unit Commanders will refer all Service members for assessment when substance use or misuse is suspected to be a contributing factor in any misconduct, such as driving under the influence/driving while intoxicated, public intoxication, drunk and disorderly conduct, spouse/child abuse and maltreatment, underage drinking, positive drug test, or when notified by medical personnel under **paragraph 2.8.3.2** of this instruction. **(T-0)** Commanders who fail to comply with this requirement place members at increased risk for developing severe substance use problems and jeopardize the mission.

2.8.2.2. Commanders will refer Service members for assessment when a problem with substance use or misuse impacting the member's job performance, interpersonal relations, physical or mental readiness, or health, is suspected, even when misconduct is not present. **(T-0)**

2.8.2.3. Commander or First Sergeant closely examine all DAF Forms 3545, *Incident Report*, for evidence of Substance-Related Misconduct, or substance misuse.

2.8.2.4. After coordination with the Staff Judge Advocate, unit Commanders will direct drug testing within 24-hours of reported aberrant or bizarre behavior, or where there is reasonable suspicion of drug use. **(T-0)** Commanders are also encouraged to ensure that Blood Alcohol Concentration (BAC) is taken as soon after a suspected Alcohol-Related Misconduct.

2.8.2.5. The Commander contacts the installation's ADAPT staff within seven (7) duty days of the misconduct to initiate the assessment process. **(T-1)** If substance-related misconduct is discovered outside the seven (7) duty day requirement, the Commander will initiate the assessment process within seven (7) duty days of notification.

2.8.2.6. Referrals for Service members in deployed locations or during a Temporary Duty (TDY) assignment (Non-Deployed Locations) will be managed IAW this instruction. **(T-1)** Given the limited resources in-theater, every effort should be made to evaluate members with substance use-related concerns prior to deployment.

2.8.2.7. Commanders will refer individuals under investigation for illicit drug use. **(T-1)**

2.8.2.8. Prior to the assessment, the Commander ensures ADAPT staff is provided any pertinent information related to the member's duty performance, on/off duty behavior, or other misconduct (e.g. observed performance/behavioral concerns, BAC results, etc.). If possible, the Commander will direct the member's immediate supervisor, or suitable alternate designee, to contact ADAPT staff prior to the assessment to provide any pertinent collateral information.

2.8.2.9. The Commander must inform the member of the following:

2.8.2.9.1. The reason for the assessment. **(T-1)**

2.8.2.9.2. The assessment is not punitive in nature. **(T-0)**

2.8.2.9.3. The member must report in uniform for the initial assessment at the appointed date and time. **(T-3)**

2.8.2.9.4. The Commander ensures the assessment and treatment of personnel is not delayed by ordinary leave or TDY assignment. **(T-0)**

2.8.2.10. The Commander is responsible for all personnel and administrative actions pertaining to members involved in the ADAPT program, to include assignment availability, promotion and retention eligibility, special duty statuses, and security clearance recommendations. Application of administrative restrictions should be based on the establishment of an unfavorable information file, or control roster resulting from the member's unacceptable behavior and not solely based on their involvement in the ADAPT program.

2.8.2.11. The Commander and/or First Sergeant will actively participate on the Treatment Team by providing input to treatment decisions. The Commander will provide command authority to implement the treatment plan when the member does not voluntarily comply with the Treatment Team's decision(s). **(T-0)**

2.8.2.12. Limited Protection for ADAPT Communications following Command Referrals. Airmen and Guardians who are command-referred for an ADAPT assessment will be granted a limited protection covering the voluntary disclosure of evidence of alcohol-related misconduct, alcohol misuse, or personal drug use or possession to an ADAPT staff member or another military medical professional with the intention of obtaining treatment.

2.8.2.13. Commanders will not use disclosures of such evidence following a command referral to ADAPT against a member in an action under the Uniform Code of Military Justice or when weighing characterization of service in a separation.

2.8.2.14. The limited protection under this section does not apply to:

2.8.2.14.1. The introduction of evidence for impeachment or rebuttal purposes in any criminal or administrative proceeding.

2.8.2.14.2. Disciplinary or other action based on independently derived evidence (other than the results of command-directed drug testing), including evidence of continued drug use after the member initially entered the treatment program.

2.8.3. Medical Care Referral.

2.8.3.1. Medical personnel will act under the assumption of not making command notifications for referrals to the ADAPT program IAW DoDI 6490.08, *Command Notification Requirements to Dispel Stigma in Providing Mental Health Care to Service Members.* Exceptions will apply to members who are under special monitoring as part of an identified program (e.g. PRAP, Presidential Support Program, etc.) as directed by specific guidance or when the provider believes there is a serious risk of harm to self, harm to others, harm to a specific operational mission, or the member is experiencing an acute condition that impairs their ability to perform assigned duties. **(T-0)**

2.8.3.2. Medical personnel will coordinate ADAPT referrals for further assessment when the member:

2.8.3.2.1. Is observed, identified, or suspected to be under the influence of drugs or alcohol during a medical appointment. **(T-1)**

2.8.3.2.2. Receives treatment for an injury or illness that may be the result of substance misuse. **(T-1)**

2.8.3.2.3. Is suspected of having a SUD Diagnosis. **(T-1)**

2.8.3.2.4. Is admitted as a patient for alcohol or drug withdrawal management. **(T-1)**

2.8.3.3. Command notification may be required for members who decline ADAPT services based on a medical referral. Medical personnel will notify command of any medical referrals submitted based on criteria listed in paragraph **2.8.3.2.1 through 2.8.3.2.4**.

2.8.4. Addictive Behavior Referrals.

2.8.4.1. Referrals for addictive behaviors must be either self-referrals or medical referrals. If a commander desires an evaluation for a non-substance related addictive behavior, and the member is not willing to voluntarily seek services, the Commander must initiate a Command-Directed Mental Health Evaluation IAW AFI 44-172. **(T-0)**

2.8.4.2. Non-substance related addictive behaviors will be treated in the Mental Health Clinic. **(T-1)** Command notifications for non-substance related addictive behaviors will be performed IAW AFI 44-172 requirements. **(T-1)**

2.9. Documentation (Prevention, Treatment, and Aftercare).

2.9.1. ADAPT staff will document the reason for referral and conduct the SUD assessment within seven (7) duty days of the referral. **(T-1)** If the member is in leave status or otherwise unavailable for assessment within the seven-day period, then the reason for the delay (e.g. incarceration, hospitalization, ARC member in civilian status, etc.) along with the anticipated date for assessment will be documented in the member's health record. **(T-2)**

2.9.2. If the member is located at a TDY, ADAPT staff at the TDY location will contact the member's Commander and home station ADAPT PM to provide information related to the substance-related event and the status of the assessment process. ADAPT staff will document any delays to the assessment. **(T-1)**

2.9.3. Documentation for prevention, treatment, and aftercare will be placed in the member's health record IAW DHA documentation guidelines. **(T-1)**

2.10. Assessment and Diagnostic Responsibilities.

2.10.1. Treatment or prevention counseling should be individually tailored IAW American Society of Addiction Medicine (ASAM) criteria and based on thorough assessment, clinical interview, collection of collateral data, and determination of risk. Treatment practices will be conducted IAW DHA-AI 6025.26, *Management of SUDs.*

2.10.2. Evaluation for specialty treatment requires a thorough biopsychosocial assessment that adheres to evidence-based Department of Veterans Affairs/Department of Defense (VA/DoD) clinical practice guidelines. Additionally, IAW DoDI 1010.04, reasonable effort should be made to include collateral information from the member's family.

2.10.3. ADAPT staff must conduct a complete review of the individual's medical history, including a thorough medication reconciliation. **(T-1)** Throughout treatment, providers should periodically monitor any changes in the patient's medical diagnoses or medication regimen to identify prescription medication misuse.

2.11. Disclosure.

2.11.1. Prior to eliciting information from the member, ADAPT staff will brief the member regarding:

2.11.1.1. Stipulations of the self-referral process, if applicable. **(T-0)**

2.11.1.2. Limits of confidentiality. **(T-0)**

2.11.1.3. Privacy Act and Health Insurance Portability and Accountability Act (HIPAA) provisions. **(T-0)**

2.11.1.4. Overview of ADAPT, patient rights, and responsibilities. **(T-0)**

2.11.1.5. The responsibilities of the respective members of the ADAPT staff engaged in assessment, education, brief preventive counseling, or treatment. **(T-0)**

2.11.1.6. The purpose, access, and disposition of medical records. **(T-0)**

2.11.1.7. The potential consequences of refusing assessment, preventive counseling, and/or treatment. **(T-0)**

2.11.2. If the referral is from the command or the result of the substance-related misconduct or concern for a substance-related health condition, ADAPT staff will explain the following to the member prior to assessment:

2.11.2.1. The requirement for command to provide additional collateral information (e.g. supervisor interview, duty performance information, etc.). **(T-1)**

2.11.2.2. The expected time requirements for the member to complete the assessment. **(T-1)**

2.11.3. When ADAPT staff are notified that a member is being considered or processed for separation and the command has informed the member of this intent, they must brief the member about their entitlement to SUD treatment with the Department of Veterans Affairs. Services will continue to be provided, according to member needs, until separation occurs. **(T-0)**

2.12. Alcohol Brief Counseling (ABC).

2.12.1. Overview. Selective prevention strategies focus on individuals who have been identified or are suspected of high-risk use of substances but do not meet diagnostic criteria for a SUD diagnosis. The goal of clinical prevention and education activities is to increase awareness and understanding of consequences related to substance use behaviors, and to develop a plan for positive individual and/or community change. Preventive counseling services should be based on an evaluation of risk.

2.12.2. ABC is conducted IAW the Alcohol Brief Counseling Manual.

2.12.3. All individuals referred to the ADAPT program who do not receive a SUD diagnosis will receive ABC. If the referral is related to the use of substances other than alcohol, ABC may be modified to address the patient's concerns. **(T-1)**

2.12.4. ABC does not constitute treatment. Qualified non-CADC technicians may be supervised by a CADC for these sessions with ADAPT PM approval. CADCs are authorized to conduct these sessions independently. Documentation for all ABC sessions require co-signature by a licensed, privileged mental health provider regardless of technician certification status.

2.13. Treatment Program Guidelines.

2.13.1. Overview. Patients diagnosed with a SUD will be placed in the appropriate level of care IAW ASAM criteria. **(T-0)**

2.13.2. At every level of care, the treatment program will reflect a multi-disciplinary approach to assist the patient to achieve full recovery, free of the negative effects of the SUD. **(T-0)**

2.13.3. Command involvement is critical to a comprehensive SUD treatment program, prevention, and early intervention stages, as well as during the aftercare stage of the ADAPT program.

2.13.4. Patients will undergo substance use monitoring during all levels of SUD treatment. **(T-0)**

2.13.5. Medically directed laboratory testing will be used to determine an individual's adherence to treatment goals. **(T-0)**

2.13.6. Patients diagnosed with an unresolved SUD will be placed on a Code 31 Mobility Restriction IAW DAFMAN 48-123, *Medical Examinations and Standards,* and AFI 48-133, *Duty Limiting Conditions*. **(T-1)** Duty and mobility restriction will be maintained until successful completion of the ADAPT program.

2.13.7. Individuals diagnosed with a SUD will refrain from the use of alcohol or other drugs during treatment. **(T-1)** During aftercare, responsible drinking goals may be considered and discussed with the entire treatment team. ADAPT staff should consider diagnostic remission requirements and clinical appropriateness prior to initiating a return to drinking protocol. A return to drinking should not be the sole reason considered for unsatisfactory progress or program failure.

2.13.8. Commanders are responsible for the administrative determination regarding the Service member and can concur or non-concur with the medical determination.

2.13.9. Involvement in self-help recovery groups (e.g. 12-step, Rational Recovery, etc.) should be used as an adjunct to treatment IAW VA/DoD CPG on Management of SUDs. The frequency of attendance is determined by the Treatment Team and the patient.

2.13.10. Patients diagnosed with a SUD will be restricted from worldwide duty for the duration of program enrollment. Determinations regarding availability for Permanent Change of Station (PCS) or TDY will be coordinated through the Treatment Team. **(T-3)**

2.14. Treatment Team Composition, Roles, and Function.

2.14.1. The primary objective of the Treatment Team is to guide the clinical course of treatment of the patient after reviewing relevant information. The Treatment Team will meet within ten (10) duty days of the initial assessment once a SUD diagnosis has been confirmed. The ADAPT PM, in consultation with the Treatment Team, makes a treatment decision(s) within ten (10) duty days of the patient's assessment. Reasons for delay(s) or absence of a defined treatment team member must be documented in the health record and conveyed to the commander. **(T-0)**

2.14.2. Membership of Treatment Team includes:

2.14.2.1. Commander. Patient's First Sergeant may be included in the Treatment Team alongside or on behalf of the Commander.

2.14.2.2. ADAPT PM chairs the Treatment Team and determines the clinical course of treatment for patients in the ADAPT program with input from the team. This role may be delegated to another privileged Mental Health Provider.

2.14.2.3. CADCs, non-credentialed mental health technicians, or privileged mental health providers involved in the case.

2.14.2.4. Medical providers. If the patient is on flight status, a flight surgeon will be included in the Treatment Team IAW DAFMAN 48-123. **(T-1)**

2.14.2.5. Primary Care Managers (PCMs) are invited to attend Treatment Team Meetings, and if not in attendance will be notified of Treatment Team outcome for continuity of care. **(T-2)**

2.14.2.6. Other individuals as deemed necessary by concurrence of the Treatment Team and with written consent of the patient. These members may include but are not limited to spouse, coworkers, or immediate supervisor.

2.14.2.7. The patient, unless deemed clinically inappropriate, or unavailable due to extenuating circumstances such as inpatient treatment or incarceration. In this case, the patient will be informed of the decisions of the Treatment Team. **(T-1)**

2.14.3. Commander and First Sergeant must be involved at program entry, termination, level of care transitions, and any time there are significant treatment concerns with the patient. ADAPT staff must brief command team on patient progress at least quarterly. If in-person contact is not feasible, consultation may be provided via virtual methods. **(T-0)** Only information pertinent to treatment decisions related to readiness and mission requirements should be shared within the Treatment Team.

2.14.4. Treatment Planning. The primary purpose of the treatment plan is to establish the framework for the patient's treatment and recovery. The treatment plan documents the level and intensity of care, incorporates issues, problem areas, life skill deficits, and goals identified during the biopsychosocial assessment, and identifies appropriate treatment resources to be utilized during the patient's course of treatment.

2.14.4.1. The treatment plan will be comprehensive, individualized to the patient's needs, and stated in behavioral terms. **(T-0)**

2.14.4.2. The treatment plan serves as the basis for determining when a patient is ready or should be moved to aftercare or requires some other level or mode of care.

2.14.4.3. Treatment plans will be reviewed at each individual session to ensure that the plan reflects status of the patient's progress toward effective SUD recovery and stabilization of other identified clinical issues. **(T-1)**

2.14.4.4. Patients will adhere to the treatment plan developed by the Treatment Team. **(T-1)**

2.15. Withdrawal Management.

2.15.1. Patients referred for inpatient treatment will be assessed to determine the level of withdrawal management services required IAW ASAM criteria. **(T-0)** When medically indicated, and when available, patient detoxification may be managed on an outpatient basis prior to inpatient or residential treatment.

2.15.2. All patients utilizing aeromedical evacuation services must have 72-hours of monitored abstinence (inpatient or outpatient) prior to departure. **(T-1)**

2.16. Continuity of Care in Treatment.

2.16.1. For patients referred to higher level of care facilities, ADAPT staff will communicate weekly to ensure continuity of care before, during, and after treatment. **(T-1)** Facility contact must be documented in the health record. **(T-1)**

2.16.2. Higher Level of Care Discharge Requirements.

2.16.2.1. Patients will be seen for a face-to-face visit no later than the next duty day. If a face-to-face visit cannot be accomplished, ADAPT staff must document the relapse prevention plan and reason for delay. **(T-1)** Treatment plan will be updated and reviewed to account for relapse prevention and suicide risk management measures. **(T-1)** Suicide risk management will be conducted IAW DHA-AI 6025.06. **(T-0)**

2.16.2.2. A Treatment Team will be conducted no later than ten (10) duty days after discharge to assess the patient's progress during inpatient or residential treatment and to design an aftercare plan. **(T-1)**

2.16.3. Transfer of Care. Patients scheduled to PCS during program enrollment, a BHDP PCS Flag will be initiated, and a warm handoff will be conducted IAW DHA-AI 6490.02, *inTransition Program*. **(T-0)**

2.17. Aftercare.

2.17.1. All patients who completed the treatment phase will participate in aftercare as part of the ADAPT program. **(T-0)**

2.17.2. The Treatment Team will collaborate on adjustments to projected aftercare treatment plan to ensure treatment, readiness, and duty requirements and accommodations are mitigated. **(T-0)**

2.17.3. Transition to the aftercare phase will be based on completion of SUD-related treatment goals and evidence of SUD remission IAW VA/DoD CPGs and ASAM criteria. **(T-0)**

2.17.4. Patients will receive individual treatment sessions by ADAPT staff at least every thirty (30) days while enrolled in aftercare to review treatment plan progression and assess program completion readiness. **(T-1)**

2.17.5. Commanders will ensure that AD personnel with a drug use history undergo monthly, random drug testing for one (1) year following their most recent discharge from a residential treatment program. **(T-0)** Those with special duty requirements may have additional drug monitoring standards imposed by professional boards or DoD policy.

2.18. Program Completion and Program Failure.

2.18.1. Program Completion. Program completion eligibility will be based on demonstrated stability of a SUD following completion of treatment and aftercare phases of the ADAPT. Utilizing VA/DoD CPGs, the Diagnostic and Statistical Manual of Mental Disorders, and ASAM criteria, the ADAPT PM collaborates with the Treatment Team to determine program completion appropriateness.

2.18.2. ARC Members. The ADAPT PM is responsible for determining successful program completion. Successful program completion or failure will be based on documentation provided by the non-military treatment provider/agency in coordination with the ARC DPH. **(T-2)** Members who have not been entered into the ADAPT Program will ensure the civilian SUD records are available for review by the Reserve Medical Unit or Guard Medical Unit. **(T-1)**

2.18.3. Program Failure. Program failure is only applicable to members who have been assessed, diagnosed, and enrolled into the ADAPT Program. Determinations regarding program failure are based on a demonstrated pattern of treatment non-compliance, recurrent substance-related misconduct, or unwillingness to engage in the treatment program. Failure decisions are determined by the ADAPT PM in coordination with the Treatment Team. If program failure is determined, the ADAPT PM will recommend command consideration for appropriate action. **(T-0)** For members determined to have failed the ADAPT program, a Duty Limiting Profile will be maintained until the SUD has been resolved through successful program completion or the member has been separated from military service. **(T-2)**

2.18.3.1. History of ADAPT completion or program failure will not be used to characterize program enrollment eligibility following a subsequent referral for SUD-related concerns, conditions, or events. Program re-enrollment will be conducted at the discretion of the ADAPT PM in coordination with the treatment team. **(T-1)**

2.18.3.2. For ANG, the ANG DPH will be notified if a decision for program failure is determined for an ANG member. **(T-0)**

2.19. Additional Treatment and Post-Aftercare Requirements.

2.19.1. Members diagnosed with a SUD will be placed on a mobility restricting profile IAW AFI 48-133, *Duty Limiting Conditions*. **(T-1)**

2.19.1.1. If the Treatment Team determines, after a period of stability in aftercare, the patient could deploy without access to follow-up services, then the profile may be removed. The final recommendation on mobility status is made by the ADAPT PM.

2.19.1.2. Aeromedical Services Information Management System (ASIMS) - Airman and Guardian Availability Management (AGAM) system will be used to initiate and monitor Service member profiles. The ADAPT PM will ensure that any mobility and duty restrictions as well as retention limiting factors are documented on an AF Form 469. Profile recommendations are completed IAW AFI 48-133, *Duty Limiting Conditions* and AFI 48-123, *Medical Examinations and Standards*. **(T-1)**

2.19.2. Flight Status. Members on Flight Status diagnosed with a SUD will be treated IAW DAFMAN 48-123, *Medical Examinations and Standards,* and the Air Force Aerospace Medicine Flight Waiver Guide Compendium. **(T-1)**

2.20. Program Evaluation.

2.20.1. ADAPT Programs will provide monthly data related to installation-specific substance-related trends utilizing the DAF ADAPT Alcohol-Related Misconduct System. **(T-1)**

2.20.2. Data analysis will identify population trends, informing targeted prevention recommendations for the CAT and Community Action Board. **(T-3)**

2.20.3. ADAPT will administer the Brief Addiction Measure to all patients at intake and at a minimum every thirty (30) days, during both treatment and aftercare to assist the DHA in monitoring program benefits and outcomes. **(T-0)**

JOHN J. DEGOES,
Lt Gen, USAF, MC, FS
Surgeon General

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

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Air Force Substance Abuse *Counselor Certification Board (AFSACCB) Governance Policy Manual*, March 2025

Air Force Substance Abuse *Counselor Certification Handbook*, March 2025

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Diagnostic and Statistical Manual of *Mental Disorders-Fifth Edition-Text Revision*, 2022

National Defense Authorization *Act for Fiscal Year 2010 (NDAA 2010),* Section 596

The American Society of Addiction Medicine Criteria: *Treatment Criteria for Addictive, Substance-Related, and Co-occurring Conditions*, Volume 1, ADULTS-Fourth Edition, 2023

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Prescribed Forms

None.

Adopted Forms

DAF Form 3545, *Incident Report*

DAF Form 847, *Recommendation for Change of Publication*

AF Form 469, *Duty Limiting Condition Report*

Abbreviations and Acronyms

ABC—Alcohol Brief Counseling

ADAPT—Alcohol and Drug Abuse Prevention and Treatment

AFI—Air Force Instruction

AFRC—Air Force Reserve Command

AFSACCB—Air Force Substance Abuse Counselor Certification Board

AFSC—Air Force Specialty Code

ANG—Air National Guard

ARC—Air Reserve Component

ASAM—American Society of Addiction Medicine

BAC—Blood Alcohol Concentration

CADC—Certified Alcohol and Drug Counselor

CAT—Community Action Team

CPG—Clinical Practice Guideline

DAF—Department of the Air Force

DAFECD—Department of the Air Force Enlisted Classification Directory

DAFI—Department of the Air Force Instruction

DAFMAN—Department of the Air Force Manual

DHA—Defense Health Agency

DHA-AI—Defense Health Agency Administrative Instruction

DHP—Defense Health Program

DoD—Department of Defense

DoDD—Department of Defense Directive

DoDI—Department of Defense Instruction

DoDM—Department of Defense Manual

DPH—Director of Psychological Health

GSU—Geographically Separated Unit

HIPAA—Health Insurance Portability and Accountability Act

IC&RC—International Certification and Reciprocity Consortium

MAJCOM—Major Command

MFM—MAJCOM Functional Manager

MTF—Military Medical Treatment Facility

PCM—Primary Care Manager

PCS—Permanent Change of Station

PM—Program Manager

PRAP—Personnel Reliability Assurance Program

SORN—System of Records Notice

SUD—Substance Use Disorder

SUDCC—Substance Use Disorder Clinical Care

TDY—Temporary Duty

VA/DoD—Department of Veterans Affairs/Department of Defense

Office Symbols

AF/JA—Air Force Judge Advocate General

AF/SG—Air Force Surgeon General

AFMEDCOM/A3HM—Air Force Medical Command/Medical Operations, Mental Health Branch

Terms

Abstinence—The practice of refraining from the consumption or use of alcohol and other intoxicating substances.

Air Reserve Component—All units, organizations, and members of the Air National Guard and the United States Air Force Reserve (USAFR).

Alcohol-Related Misconduct—This type of conduct includes driving while intoxicated, public incidents of intoxication and misconduct, underage drinking, or similar offenses.

Centralized Credentials Quality Assurance System (CCQAS)—An online application for credentials, privileging, adverse action and risk management database utilized within Department of Defense.

Certified Alcohol and Drug Counselor (CADC)—Mental Health technicians who serve in clinical roles within the ADAPT Program. CADCs can also be officers. They provide services in the following 12 core functions outlined by the IC&RC.

Clinical Treatment—Services designed for the treatment of patients diagnosed with a substance use disorder. These services include a wide range of programs including Intensive Outpatient, Partial Hospitalization Program, residential and inpatient programs.

Detoxification—A planned management of alcohol and drug withdrawal. Patients usually undergo medical detoxification an inpatient setting.

Drug—Any controlled substance included in Schedules I, II, III, IV, and V in 21 U.S.C. 812, including anabolic or androgenic steroids, or any intoxicating substance other than alcohol, that is inhaled, injected, consumed, or introduced into the body in any manner to alter mood or function.

Eyes-on supervision—Direct contact with the patient by a licensed privileged provider for sufficient length and interaction, generally after the Mental Health Technician (MHT) or CADC assesses the patient, to validate the assessment and recommendations before the patient departs the appointment.

Intoxication—A condition that follows the administration or consumption of a psychoactive substance causing disturbances in the level of consciousness, cognition, perception, judgment, affect or behavior, or other psychophysiological functions and responses.

Privileged Mental Health Provider—Military (Active or Reserve component) and civilian personnel (civil service and providers working under contractual or similar arrangement) granted privileges to diagnose, initiate, alter, or terminate regimens of healthcare within defined scope of practice.

Relapse—A return to drinking or drug use after a period of abstinence.

Selective prevention (from Institute of Medicine)—For those who may have a higher likelihood of developing unhealthy substance use habits as a result of particular risk factors.

Substance—Alcohol and other mind- or mood-altering drugs, including illicit drugs, prescribed medications, and over-the-counter medications.

Substance Use Disorder—As defined by the Diagnostic and Statistical Manual of Mental Disorders.

Substance or Drug Misuse—The use of any substance with or without a prescription with the primary goal to alter one's mental state. Misuse includes the use of any substance in a manner that inhibits major areas of life functioning, to include duty impairment.